

CHAPTER 9

Finding, Focusing, and Fleshing Out Protectors

In IFS training we take six steps with protective parts, which are designated the 6Fs: *find*, *focus*, and *flesh out* followed by *feel toward*, *befriend*, and *explore fears* (Anderson et al., 2017). In this chapter we focus on how to help the client find a target part, focus on it, and flesh it out. We conceive the last instruction loosely, since parts can show up kinesthetically or aurally, and when they do show up visually they are just as likely to look like a geometric shape or cloud as they are to appear in human form. Regardless of how our parts appear to us, when we *go inside* (pay attention internally), we quickly enter a liminal state in which we are aware of our surroundings but feel as if we are in another world. While the client drops into this state, the Self-led therapist does too, and from this place may intuit a lot about the client's inner system. However, our job is to ask not tell, so we guide the client in exploring her system and all that information unfolds organically.

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WHEN PARTS RESIST THE LANGUAGE OF PARTS

Although many clients take to parts language easily, some have parts who resist. They may fear being labeled *crazy* because our culture has long pathologized multiplicity; they may not want to focus on or reveal what goes on inside; or they may be concerned about being coerced into a particular view of things by the therapist. Additionally, some clients are simply dominated by a part who will not cede ground on any topic until it feels heard and is convinced the therapist understands its viewpoint: "Don't talk about my parts! The problem is Stella messing up my bathroom." Parts who are in this frame of mind will happily wage a turf war over language. Our advice is simple: Don't

t get pulled in. Regarding language, the easiest option is to use whatever language the client uses—whether this is a *feeling*, a *thought*, an *impulse*, a *pain*, etc.—until she feels more comfortable with you and with focusing inside.

THREE COMMON BEGINNER ERRORS WITH LANGUAGE

Beginners in IFS are liable to make three mistakes when it comes to introducing the concept of parts (see [Box 9.1](#)). The first error involves talking about parts too eagerly and using too many words, which elicit caution in the client's managers. When therapists' managers feel discouraged by this caution, they may then push their therapists to give up or sell the model even more vigorously. The second error is to be so tentative and inconsistent with the model's language that the client's managers sense danger and resist. The third error involves anticipating that clients will find parts language silly or bizarre and communicate this unease, which also elicits caution in therapists' managers. The more confident and even playful the therapist is about the model and the language, the more likely the client will participate willingly. The catch, of course, is that therapists need time and experience in order to feel confident. This is one reason IFS training is largely experiential. It is also why we recommend having the experience of being a client with a skilled IFS therapist.

BOX 9.1. Beginner Errors with Language

- # Talking about parts too eagerly and using too many words, and then responding to resistance by either giving up or selling the model more vigorously.
- # Speaking so tentatively and inconsistently with the language of parts that the client's managers sense danger and resist.
- # Anticipating that clients will find parts language silly or bizarre and communicating this unease.

GOING INSIDE: WINNING HEARTS WITH QUESTIONS

Once clients agree to try the model, with or without using the word *parts*, we have several options. Probably the least threatening is simply to ask questions. As we assess the client's problems and identify different parts, we can inquire about each part's relationship with the client's Self, with other parts, and with the people who surround the client. The goal is to get a sense of the client's inner and outer ecologies. If you are a beginner at IFS, we recommend taking time with this step. Identifying the client's parts and being aware of the client's dominant relationships will help you

enter his system safely. When the client's managers seem extremely mistrustful or afraid, even experienced IFS therapists who trust their intuition on pacing and can easily spot common relational patterns will want to talk *about* parts for a while before helping the client talk *to* them. But once the client is used to the language of parts and signals openness to exploring further, the next step is to find a target part.

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FINDING A TARGET PART

At the outset of therapy we elicit a lot of information about the client's system. We can illustrate all this on a white board and look at it together with the client, we can take notes on paper, or we can rely on memory. However we track this initial information, it's useful to keep summarizing what we hear: *You mention three parts. There's one who wants to move to Oregon, one who wants to apply to graduate school, and one who just wants to stay here and get a job right now to save money. Is that right?* If the client endorses our summary, we move on to choosing a target part by asking another question, *Who needs your attention first?*

FOCUSING ON AND FLESHING OUT THE TARGET PART

To find a target part, we ask the client to focus inside, observe, and then describe the part (or flesh it out). For some clients, finding, focusing, and fleshing out happen almost simultaneously in a quite unremarkable way. But for others, like those whose subjective experience has been chronically invalidated by authority figures, noticing and describing parts are novel and exciting experiences. Many people who have been persistently dissociated have only a vague sense of their parts when they start therapy, but they still gain a lot from questions that direct them to observe more closely. Curiosity and kindness go a long way toward inviting parts to risk showing up. At the same time, the therapist can begin to get a sense of the client's internal experience, which can vary from chaotic and restless, to highly regimented and slow, to clear and easily navigated.

LOCATING THE TARGET PART IN A SENSATION, FEELING, OR THOUGHT

If a client doesn't immediately know which part needs attention first (or, in an ongoing therapy, if no part from the week before is waiting for the client to return), we can ask her to focus on a sensation,

feeling, or thought, which are all portals to the inner world. Following are four examples of locating a target part that range from seeing, hearing, and feeling the part to just being able to sense the presence of a part.

Leigh and the Head Hammerer

Leigh needs some guidance to pay attention internally and notice her head-hammering part and pay attention to what he is saying.

THERAPIST: Where would you like to start?

LEIGH: I don't know.

THERAPIST: Take a moment to focus inside. What do you notice?

[The therapist guides Leigh to focus internally, which most suffering people avoid without being aware that they are avoiding.]

LEIGH: The usual. I'm no good.

[Before paying attention internally, Leigh is globally aware of the message her inner system takes from the head hammerer's behavior, but she has not actually listened to him much less engaged with him.]

THERAPIST: How does the part who says you're no good show up?

[The therapist immediately moves from the global to the relational and specific with parts language, framing Leigh's experience as the communication of a part.]

LEIGH: I guess I hear it and my head hurts.

THERAPIST: Want to start with that?

[The therapist asks for permission to pay attention to this phenomenon.]

LEIGH: Hmm. I want to be sure I do this right.

[And a part who is anxious to comply with external expectations immediately shows up.]

THERAPIST: Since there is no right way, you can't do it wrong. You might sense a part and know it's there. Or you might see the part, hear it, or feel it physically. Our job is to pay attention to your experience and notice who's there.

[The therapist makes a plug for noticing, which is a crucial step toward unblending.]

LEIGH: Oh now I see the guy all right! He's got a sledgehammer, which he slams around in my head like a bell. He says: *There! There! This is what you deserve. Why don't you listen to me?*

Leigh's head hammering manager is graphic and vivid. Now she not only sees him, she hears him and feels the effects of his hammering inside her head. Notably, he is not saying *You're no good* as her parts interpreted the message; rather, he is saying *Why don't you listen to me?*

Noah Can't See Anyone

In contrast, at the outset of his therapy, Noah can't see his parts. Only time can tell if this will change. As mentioned previously, a small percentage of people, myself (RS) included, hear or sense parts rather than see them. Noah may not have internal vision. On the other hand, his target part could be too afraid or too blended to be seen or a manager could be blocking it. If so, Noah's inner process will become visual later on. Either way the therapy can proceed.

NOAH: I want to focus on anxiety.

THERAPIST: Where do you notice it?

[The therapist immediately moves to the client's embodied experience of the emotion he is noticing. Parts affect the body and show up in the body.]

NOAH: In my chest.

THERAPIST: Is it okay to focus on that?

[The therapist asks for permission to notice the part, without yet calling it a part.]

NOAH: Yes.

THERAPIST: How do you notice the anxiety?

NOAH: It's a sensation in my stomach that radiates out all over my body.

THERAPIST: Do you see anything when you focus on it?

[Since the inner eye offers a quick connection to parts, the therapist checks to see if Noah is internally visual.]

NOAH: No.

THERAPIST: How do you feel toward it?

[The therapist moves right on to this standard parts-detecting question.]

NOAH: Oh god, I'm so sick of it!

THERAPIST: Would the parts who are sick of it be willing to let you help it?

[The therapist shifts into parts language now, still not having named anxiety as a part.]

NOAH: Well sure, if I knew how. I guarantee you I've tried.

THERAPIST: If they're willing, I'll show you something different.

[The therapist speaks to the willingness of Noah's protectors, knowing they are listening.]

NOAH: This has been around my whole life. Nothing helps with anxiety.

[Noah speaks from a resigned, hopeless part.]

THERAPIST: You've felt this way for a long time. And you've tried other things that haven't helped.

So it makes sense that you have parts who don't believe me. But would they be willing to let you try something new?

[The therapist validates and asks permission to proceed.]

NOAH: No.

THERAPIST: Can you ask why?

[Without arguing, the therapist is curious about protector fears.]

NOAH: I'm hearing that hope isn't all that helpful.

[And a protector reveals its fear.]

THERAPIST: What happens when you feel hopeful?

NOAH: I get depressed.

THERAPIST: What makes you depressed?

NOAH: Disappointment.

THERAPIST: So if you feel hopeful and then disappointed, you get depressed, is that right?

[The therapist sums up what they have just done: traced the apparent problem—anxiety—back to its roots in a depressed part.]

NOAH: Yes.

THERAPIST: Would it be okay to help the part who gets depressed in response to disappointment?

NOAH: I guess so.

THERAPIST: Check inside and ask if that would be okay.

[I guess so is equivocal. Rather than going with this, the therapist sticks with the process of listening to protectors until they give affirmative permission.]

NOAH: I'm sensing a lot of reluctance. They say okay but reserve the right to stop us any time. They don't want the depressed part taking over.

THERAPIST: I don't either. Let's ask it if it would be willing to not take over in return for having your attention.

[The therapist validates this protector's concerns and proposes to negotiate unblending with the exile as a way forward.]

NOAH: I'm hearing Yes.

THERAPIST: So is it okay to go ahead with the depressed part?

[The therapist checks again for permission to proceed.]

NOAH: Yes.

THERAPIST: Before we do that, I have one question. Is that okay?

NOAH: Yes.

THERAPIST: Is the anxious part the one who gets disappointed and then feels depressed?

NOAH: Yes.

As we hear, Noah has experienced anxiety since childhood and nothing he's done so far has helped to lower it. He does not see the anxious (disappointed, depressed) part—he feels it. Although other parts are frustrated with this anxiety, they are not willing to let Noah feel hopeful about trying something new in relation to it because failure and disappointment, which have occurred in the past, activate its depression. Understanding all this, Noah and his therapist can negotiate with the anxious part not to overwhelm so that Noah's Self is present and his protectors feel safe proceeding.

Elijah Dances with His Exile

For a third example, here is a client who senses and moves with his parts, which he sees only vaguely as shadows. Elijah is a 25-year-old dancer. He is gay and his parents are fundamentalist Christians who live in a homophobic community. He came to therapy because he wanted to convince them, as he says, to “step back” from their prejudices and be his parents. He describes them as good, devoutly religious people who are active in their congregation, and he firmly believes that they want to reconnect with him, their only child. At the same time, some of his parts are afraid of them.

ELIJAH: My parents can be mean.

THERAPIST: So who needs your help?

[The therapist immediately guides Elijah in what we call a U-turn, so that he will focus internally on his parts (a perspective that prompts his parts to unblend from his Self), rather than encouraging his parts to focus on his parents (from which perspective they are most likely to stay blended).]

ELIJAH: I don't want the fear to take over. I know they're more than that.

[Elijah names a part.]

THERAPIST: Do you want to help this fearful part then?

ELIJAH: Yes.

THERAPIST: Where do you notice it?

[The therapist invites Elijah to locate the part physically.]

ELIJAH: In my gut.

THERAPIST: Can you see it?

[The therapist checks on how he perceives his parts.]

ELIJAH: Like a shadow. More I feel it.

THERAPIST: How do you feel toward it?

[The therapist checks for Self-energy.]

ELIJAH: Compassionate.

THERAPIST: How does it respond?

ELIJAH: It's despondent.

THERAPIST: What does it need from you?

[Since Elijah has good access to his Self, the therapist invites the part to state its needs.]

ELIJAH: I think we'll just move together.

THERAPIST: Do you want to move in this room?

ELIJAH: *(closing his eyes)* No. We'll move together inside.

[Elijah's Self is in charge now.]

THERAPIST: *(after a few moments)* How's it going?

[The therapist checks in. We decide whether to check in by observing the client and sensing when help might be needed. Clients who are in the middle of a deep process can feel interrupted by this. If so, we want them to feel free to say so. If the client asks not to be interrupted, it's good to check in later and find out what works best for this client's system. At the same time, dissociative parts can take over without much of an external signal, so it's wise to err on the side of checking in with new clients or those who have flagged dissociation as a concern.]

ELIJAH: This part likes being with me.

THERAPIST: Will it trust you?

ELIJAH: Me, yes—but my parents, no.

THERAPIST: Will that work?

[Again, the therapist follows the lead of Elijah's Self.]

ELIJAH: I think I have to spend more time with this part before I do anything. It's not ready. I won't contact them until it is ready.

As we hear, Elijah comes to therapy with a specific goal and plenty of Self-energy, but this goal scares a part who has felt hurt by his parents. He has compassion for the part and finds a way to be with it, which involves the part and his Self moving together internally.

A Wall Blocks Toshi's Vision

And, finally, here is a fourth example of inner communication that involves a 45-year-old woman feeling and then hearing from a part whom she doesn't see until she wins more cooperation from a wary protector.

TOSHI: Someone's there. But I can't see anything.

[This statement is a clue that Toshi may usually be internally visual.]

THERAPIST: How do you notice it?

TOSHI: I just know it's there. But it's like the lights are out.

THERAPIST: How do you feel toward it?

[The therapist defaults to our basic parts-detecting, Self-energy-detecting question.]

TOSHI: I would like to meet it.

THERAPIST: How does it respond?

TOSHI: Blank.

THERAPIST: Would it be okay to feel into the blankness?

[All parts are welcome and all experiences, even blankness, are germane as communications from parts.]

TOSHI: I hear a little voice saying, *Wall!*

[As is often the case, parts reward attention and patience with more direct communication.]

THERAPIST: There's a wall?

TOSHI: Yes. When I asked the wall to move it said, *Who are you?*

THERAPIST: How do you feel toward the wall?

[Having noticed two parts, the wall (who Toshi experienced at first as blankness) and the part who said Wall!, the therapist moves on to the parts-detecting question to find out if other parts are reacting to the wall and to assess Toshi's level of Self-energy.]

TOSHI: Well, it must have some reason to be cautious ... now it's softening a little. I'm telling it I'm here to help.

[Toshi's Self is immediately evident in her response.]

THERAPIST: (after a silence) How does it respond?

[Although Toshi has plenty of Self-energy, her long-ish silence at this point is ambiguous. The therapist, who does not know if Toshi is hearing from the wall now or if something else has happened, decides to check in.]

TOSHI: It turned transparent. I see a child-size person behind it.

THERAPIST: What needs to happen?

[Since Toshi's Self is present and things are moving along internally, the therapist stays out of the way.]

TOSHI: The wall doesn't want me to be overwhelmed by this child.

THERAPIST: I agree. But we can just ask the child not to overwhelm you. If it says yes, would the wall let you help the child?

[Here the therapist takes the opportunity to validate and address the concerns of the wall, who is a protector.]

TOSHI: Yes.

As we see, Toshi is first able to notice a part without seeing it. Then she discovers a wall between her and this unseen part. Toshi has enough Self-energy to reassure the wall, and in return it explains that it fears emotional overwhelm, the number-one concern of protectors. When the therapist validates this concern and explains how to avoid overwhelm, the wall agrees to cooperate.

As these four examples illustrate, there are many ways of perceiving and communing with parts. For some people inner communication includes visuals, sounds, and sensations; for others it includes just one of these modes. Clients who do not see their parts often need reassurance that subjective experience is highly idiosyncratic and varies widely.

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CONCLUSION

Find, *focus*, and *flesh out* are the first three steps of the IFS inquiry. Depending on how dissociative the client has been in her life and how much Self-energy she can access, these steps may conflate and go by in a flash, or they may be quite distinct and take time. After we introduce the concepts and language of IFS, we check for permission from managers (*Is it okay to notice what's going on inside?*). Once we get a green light, we enter the client's inner world and locate a target part by noticing the client's body, feelings, and thoughts. If the target part is a manager, we can safely proceed with detailed questions that help clients inquire into and articulate their inner experience.

However, if the person is in the middle of an exile jailbreak and is having trouble functioning, this kind of inquiry is problematic. We urge therapists to be mindful and Self-led when taking a client inside for the first time. It is safe to ask questions that flesh out parts if the client is able to focus without quickly veering off and changing the subject. Assuming it is safe to proceed, we may find that the client experiences the inner world visually, and that these visuals become more vivid for having been noticed. Other clients see inside only vaguely. And yet others experience parts aurally

or kinesthetically. Just as all parts are welcome in IFS, all ways of experiencing are equally valid and potentially profound.